

OPEN

The Lived Experience of Medication for Opioid Use Disorder

Qualitative Metasynthesis

Aimee Techau, MSN, PMHNP-BC, CARN-AP ○ Emily Gamm, LCSW, CAS, ACS ○

Mia Roberts, MSN, CPNP-PC ○ Lorraine Garcia, MSN, CNM

Abstract

Purpose of the Study: Buprenorphine and methadone, when used as maintenance therapy in opioid use disorder (OUD), can significantly reduce the risk of death after an opioid overdose. Despite the many benefits, medication for OUD (MOUD) remains controversial and underutilized. The aim of this study was to (a) identify studies that explicate the lived experience of MOUD, (b) conduct a methodologically sound critical quality assessment, (c) conduct a metasynthesis of identified qualitative studies, and (d) analyze the results through the lenses of critical social justice, feminist standpoint, and social theories to inform a more culturally responsive, effective, and holistic approach to treatment to increase the utilization of and receptivity to MOUD in the treatment of OUDs.

Design and Methods: Metasynthesis is the deliberate process of synthesizing data from multiple qualitative studies to create a new interpretation of previously published research. Our metasynthesis focused on forming a new interpretation of the lived experience of MOUD. Of the 292 studies identified, eight met inclusion criteria. The overall quality of the studies was variable. The study population included adults aged ≥ 18 years with OUD or dependency.

Results: Metasynthesis produced three overarching themes: (a) The duality of MOUD creates ambivalence in how a person experiences MOUD; (b) the structure of treatment and systemic influences matter, and (c) MOUD as a stepping-stone with multiple meanings in recovery.

Implications: Understanding experiential dualities allows professionals to see from other perspectives and can

inform interventions and treatment structures as well as serve as a backdrop for social change. The synthesized new meaning of MOUD was an ambivalent experience influenced by intersectionality.

Keywords: Help-Seeking Behaviors, Lived Experience, Medication-Assisted Treatment, Medication for Opioid Use Disorder, Opioid Use Disorder, Qualitative Metasynthesis, Recovery

INTRODUCTION

The opioid crisis is one of the worst public health emergencies in the history of the United States (Hodge et al., 2018), and opioid overdoses now surpass car accidents as the leading cause of death (American Society of Addiction Medicine, 2016). Furthermore, the COVID-19 pandemic caused a surge in overdose deaths in practically every state during the first 8 months of 2020, representing the largest year-over-year increase in fatal drug overdoses in the last two decades (American Medical Association, 2021). Fueled in part by opioid deaths, all-cause mortality rates in middle-aged White Americans continue to rise and now exceed levels last seen at the beginning of the 20th century, starkly contrasting with other industrialized nations, which have been experiencing yearly improvements in this domain (Case & Deaton, 2017).

The opioid crisis is believed to have originated with the overprescribing of opioids, primarily for pain, in the 1990s (U.S. Department of Health and Human Services, 2016), and the Centers for Disease Control and Prevention (2017) estimates that opioid prescriptions have nearly quadrupled since 1999. Although America represents “less than 5% of the world’s population, it consumes roughly 80% of the world’s opioid supply and 99% of the global hydrocodone supply” (Manchikanti & Singh, 2008, p. 63).

Opioid use disorder (OUD) is defined as the problematic use of opioids, whether prescribed or illicit, marked by withdrawal and tolerance and meeting two or more criteria that show diminished health and social functioning over a 12-month period (American Psychiatric Association, 2013). Pharmacological treatments for OUD include buprenorphine (BUP) and methadone (MET), which, when used as maintenance therapy to address OUD, can significantly reduce the risk of death after

University of Colorado College of Nursing, Aurora.

The authors report no conflicts of interest. The authors alone are responsible for the content and writing of the article.

This is an open-access article distributed under the terms of the Creative Commons Attribution-Non Commercial-No Derivatives License 4.0 (CCBY-NC-ND), where it is permissible to download and share the work provided it is properly cited. The work cannot be changed in any way or used commercially without permission from the journal.

Correspondence related to content to: Aimee Techau, 516 Sage Grouse Circle, Castle Rock, CO 80109.

E-mail: Aimee.techau@ucdenver.edu

DOI: 10.1097/JAN.0000000000000475

an opioid overdose. A study found that “compared to those not receiving medication-assisted treatment, opioid overdose deaths decreased by 59% for those receiving methadone and 38% for those receiving buprenorphine over the 12-month follow-up period” (Larochelle et al., 2018, p. 137). Despite the growing body of evidence that supports the use of BUP and MET to assist individuals recovering from OUD, these medications remain mostly underused: “Less than one-third of patients received any medicine for OUD in the first year following an overdose, including MET (11%) and BUP (17%)” (Larochelle et al., 2018, p. 137). The benefits of medication for OUD (MOUD) extend beyond preventing overdose deaths; a systematic review revealed increased rates of retention in treatment, decreased rates of incarceration and hospitalization, and a positive impact on HIV and Hepatitis C risk behaviors (Connery, 2015).

Most studies of patient experiences have been quantitative, evaluating likeability, retention, and the ability of MOUD to promote abstinence and prevent overdose and death. However, quantitative research minimizes patients' perspectives, and qualitative research could help determine why such treatment remains both underutilized and controversial (Lindgren et al., 2015). This study therefore aims to identify research that explicates the lived experience of patients undergoing MOUD and how that experience influences their recovery, help-seeking, and treatment; to perform a methodologically sound critical quality assessment; and to conduct a metasynthesis to offer a novel, integrated, and interpretive analysis of the lived experience of individuals with OUD, grounded in current qualitative evidence. A subsequent aim was to analyze the results through the lenses of critical social justice, feminist standpoint, and social theories to inform a more culturally responsive, effective, and holistic approach to treatment to increase the utilization of and receptivity to MOUD in the treatment of OUDs. A conversation about the lived experience of MOUD—and substance use treatment more generally—can be situated at the juxtaposition of modern and postmodern philosophies.

MODERNITY VERSUS POSTMODERNITY

Scientific, philosophical, and methodological approaches to medicine and the treatment of SUDs have been rooted primarily in modern philosophical constructs and in an approach that is atomistic and characterized by Newtonian science. As Farmer et al. (2006) stated, finding the molecular basis for disease is the “holy grail of modern medicine.” In modern Cartesian epistemology, objective knowledge formed through methodological doubt at the “molecular” level, then built upon, results in a truth that can be applied to all persons, in all places, and at all times and thus requires no consideration of biosocial systems.

Conversely, postmodern philosophy—particularly feminist standpoint theory as an epistemological construct—considers persons within their contexts or lived experiences, calling the atomism of Descartes and Newton into question. In postmodern constructs, therefore, cultural and identity factors are inextricably tied to what we know, so attaining universal knowledge is not possible and is ultimately the wrong aim. Feminist social epistemologies are also driven by how

knowing occurs in the context of oppression and systemic power dynamics (Anderson, 2020).

The social theory of civil rights activist W. E. B. DuBois stands in the gap between modern and postmodern philosophies. Arguably the greatest public sociologist of the early 20th century and born of the modern philosophical era, DuBois shifted the atomistic conception of self to one that is culturally embedded and socially constructed—a social construction that ultimately exposes and has implications for systems of oppression and can thus be the impetus for a social and ideological change that has notable significance for more effective substance use treatment (Meer, 2019).

METHODS

Search Strategy

A. Techau—one of the authors and a psychiatric and addictions nurse practitioner—conducted the literature search. Studies were identified by searching four electronic databases

Search Terms for Lived Experience, Qualitative, & Medication Assisted Treatment

The following search query was for PubMed, Embase, PsycINFO, and Google Scholar.

Lived Experienced Terms

“Help-Seeking Behavior”[Mesh] OR “Help-Seeking Behavior” OR “Help-Seeking Behaviors”)) OR “lived experience” OR “Perception”

*Help-seeking associated with lived experience as this is how recovery is conceptualized. No MeSH term for lived experience.

Qualitative Terms

“semi-structured”[TIAB] OR semistructured[TIAB] OR unstructured[TIAB] OR informal[TIAB] OR “in-depth”[TIAB] OR indepth[TIAB] OR “face-to-face”[TIAB] OR structured[TIAB] OR guide[TIAB] OR guides[TIAB]) AND (interview*[TIAB] OR discussion*[TIAB] OR questionnaire*[TIAB]) OR (“focus group”[TIAB] OR “focus groups”[TIAB] OR qualitative[TIAB] OR ethnograph*[TIAB] OR fieldwork[TIAB] OR “field work”[TIAB] OR “key informant”[TIAB])) OR “interviews as topic”[Mesh] OR “focus groups”[Mesh] OR narration [Mesh] OR qualitative research[Mesh])

Medication-Assisted Treatment

“Opiate Substitution Treatment”[Mesh]
((“Buprenorphine”[Mesh] OR “Buprenorphine, Naloxone Drug Combination”[Mesh]) OR “Methadone”[Mesh] OR “Biodone” OR “Dolophine” OR “Metadol” OR “Metasedin” OR “Symoron” OR “Methadone” OR “Methadose” OR “Methex” OR “Phenadone” OR “Physeptone” OR “Phymet” OR “Pinadone” OR “Amidone” OR “Methaddict”) OR “Medication assisted treatment”Note: MeSH = Medical Subject Headings

—PubMed, Embase, PsycINFO, and Google Scholar—for the key phrases “medication-assisted treatment,” “medication for opioid use disorder,” and “lived experience” (Box 1). XX also used a qualitative MeSH algorithm designed by the University of Washington's Health Sciences Center to improve the extraction of qualitative studies, and an academic librarian was consulted to ensure the accuracy of the search terms. The database searches were supplemented by citation chaining, author searching, reviews of gray literature and of physical copies of the *Journal of Addictions Nursing*, and repetition of the initial search strategy before finalizing the study (Sandelowski & Barroso, 2006). The new MeSH terms generated for “medication-assisted treatment,” “lived experience,” and “recovery” were merged, and the search was limited to peer-reviewed, English-language, human studies on adults aged 18 years or older with OUD or dependency, excluding articles indexed as clinical trials. For added reassurance, the qualitative search filter was also applied. EndNote X9 was used to store and manage the study information and to remove duplicate citations.

Study Selection and Appraisal

Inclusion and Exclusion Criteria For the purposes of this study, MOUD included pharmacotherapies, such as MET and BUP. No distinction was drawn between BUP and BUP with naloxone. Lived experience was defined, according to Given (2008), as the representation and understanding of a research participant's experiences, choices, and options and how these factors influence their perception of knowledge. The inclusion criteria were as follows: published studies that (a) included adults aged 18 years or older with an official diagnosis of OUD or dependency; (b) focused on the lived experience of individuals receiving MOUD; (c) employed qualitative methods; (d) were retrieved between September 21, 2019, and October 1, 2021; and (e) were published or completed between 2002, when BUP became available in the United States, and April 4, 2021. Allowances were made for changes in the official diagnostic criteria and language between the fourth and fifth editions of the *Diagnostic and Statistical Manual of Mental Disorders*. The exclusion criteria were as follows: published studies that (a) reported quantitative descriptive research, including surveys and focused measurements of cravings, medication satisfaction, and negative urine toxicology reports; (b) reported on the detoxification of opioids and MOUD; (c) used atypical representations of the disease or comorbid medical conditions, such as human immunodeficiency virus (HIV) and hepatitis C virus; (d) reported purely verbal data with surface-level analysis (Sandelowski & Barroso, 2006); (e) were not published in English; or (f) were dissertations or published in non-peer-reviewed journals. No exclusion criteria were established regarding treatment settings or geographical location, as diverse settings are important for new discoveries.

Article Adjudication The three-member team used triangulation to determine which articles identified in the literature search were relevant and met the inclusion criteria.

A Critical Review of the Methodology The McMaster University Tool was selected for this study and used to evaluate the qual-

itative research articles based on the presence of 17 critical components. To determine their quality, each study's purpose, literature review, study design, sampling methods, data collection methods, descriptive clarity, analytic rigor, auditability, and overall rigor were assessed (Goins et al., 2014). The robustness of each study was further evaluated using four additional components of trustworthiness: credibility, transferability, dependability, and confirmability (Goins et al., 2014).

Metasynthesis The performance and reporting of the metasynthesis were guided by international standards and the work of Goins et al. (2014). Primary studies were divided and triangulated by the three-member team, all of whom are advanced practice nurses with formal course work in qualitative methods and an interest in MOUD; their specialties are behavioral health, pediatrics, and midwifery. XX, a psychiatric and addictions nurse practitioner, evaluated all nine studies selected, whereas the other team members evaluated four and five studies. Iterative team discussions were held via Zoom, during which the themes and subthemes were discussed using an inductive approach. Each member created a taxonomic structure of themes, concepts, and relationships based on Sandelowski et al. (2007) descriptions.

Team members independently identified key quotations to support themes and subthemes. An audit trail was maintained by recording the Zoom session, documenting the taxonomic structures, and taking notes during discussions. The process was paused any time a team member noticed conflicting content, lack of clarity, or the introduction of coding that came from only one member, and discussion continued until a consensus was achieved and a new thematic understanding of the lived experience of MOUD participants emerged.

Selection Process

Figure 1 illustrates the article selection process, with the number of articles eliminated at each step. The search yielded 292 studies across four databases; the additional methods yielded no additional studies. After removing duplicates ($n = 122$), 170 unique studies were identified, of which 127 were rejected after screening titles and 16 after screening abstracts, leaving 25 studies for full review. After the critical review, only nine studies were relevant and met the inclusion criteria. Most articles that were excluded focused on the application of MOUD in patient detoxification for opioids, reported purely verbal data with surface-level analysis, used mixed methods, or reported on lived experience with comorbidities, such as HIV and hepatitis C virus. The accepted studies used interviews ($n = 5$), focus groups ($n = 3$), or both ($n = 1$).

Study and Participant Characteristics Table 1 lists each article's authors, publication year, study purpose, location, study design, methods, and participants and a summary of its findings. The studies were conducted in North America ($n = 4$), Norway ($n = 2$), Australia ($n = 2$), and Sweden ($n = 1$) between 2012 and 2018, but they did not consistently report on study time frames. Sample sizes ranged from 7 to 77 participants, and the prevailing design was qualitative descriptive research. The qualitative data from the nine studies represented 271 adults with OUD or dependency undergoing MOUD. The

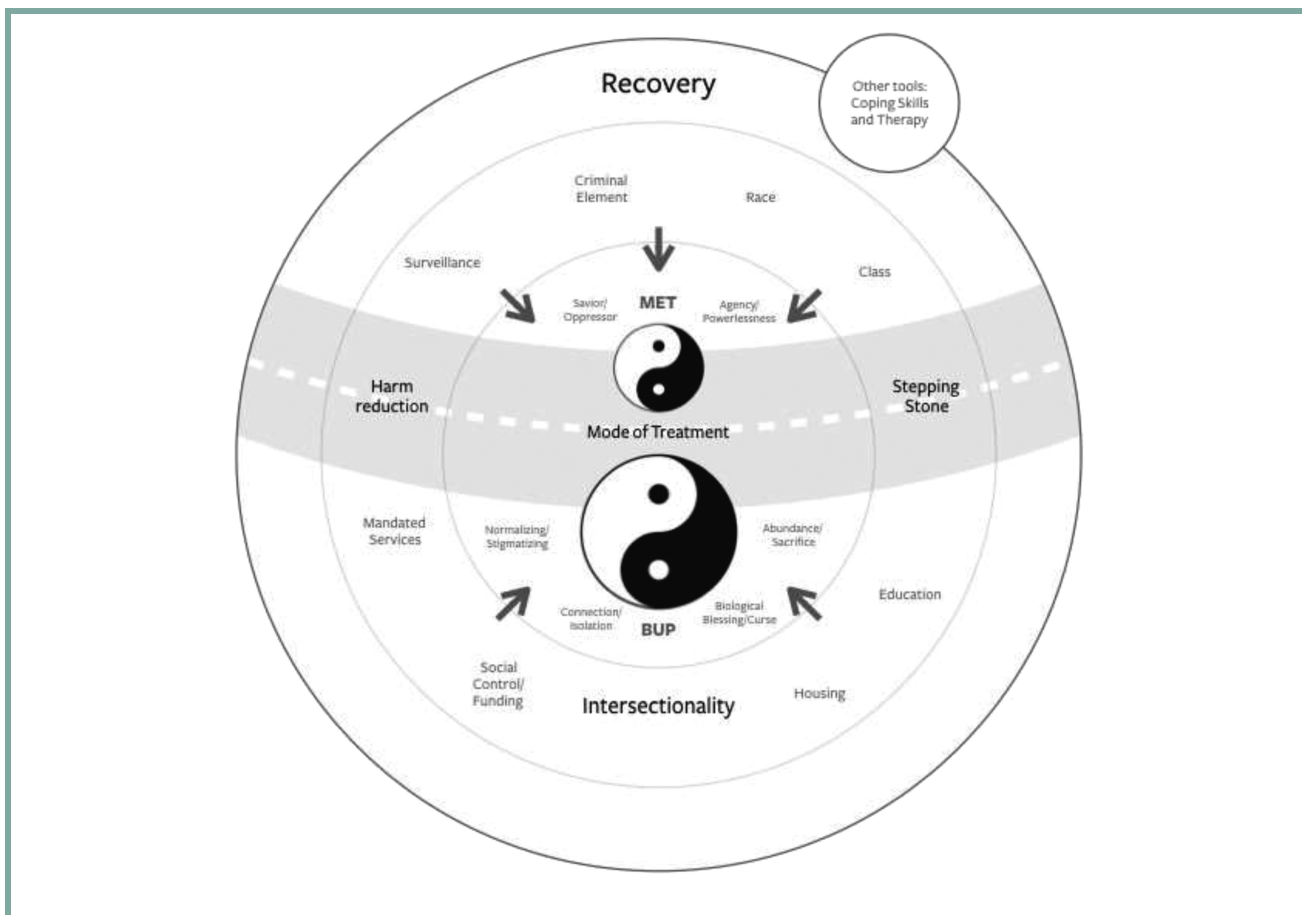


Figure 1. Article selection process.

gender of the participants was reported in all but one study—141 participants were male and 58 were female—and their ages ranged from 18 to 82 years. More than half of the studies documented participants' races and ethnicities, which included African American, Asian American/Pacific Islander, Latinx, Native American, other mixed, Aboriginal, Torres, and White. Studies that reported socioeconomic and educational status found that most participants had a high school education or less and were unemployed or on disability. Four of the studies reported the duration of MOUD, with less than 28 days being the shortest and more than 10 years being the longest. Most studies were inconsistent in reporting the specific breakdown between MET and BUP.

CRITICAL REVIEW OF THE METHODOLOGY

All studies featured a purpose and/or research questions and a review of relevant literature. Three of the studies identified a theoretical perspective, including the theories of social stigma (Goffman, 1986), bare life (Agamben, 1998), and ecological systems (Bronfenbrenner, 1992). The sampling methods used appeared to capture the desired populations and provide adequate diversity in the data. Three of the studies reported on data saturation and the sample size chosen, and all the data

analyses were inductive, implying analytic rigor. Few of the studies described an audit trail. The findings appeared consistent with the data, generating rich and meaningful portrayals of the phenomena being studied. Some components of overall rigor were present, and all the studies addressed transferability. Overall, it was found that seven of the studies were credible and transferable, but they were not always confirmable.

THEMES

Conceptual themes and subthemes were identified in the studies by combining and using units or common and unique features (Goins et al., 2014). Table 2 illustrates the three analytic themes and the subthemes showing a new meaning of MOUD, in which it is viewed as an ambivalent experience influenced by intersectionality. Ambivalence is defined as having mixed emotions (negative or positive) at the same time, whereas intersectionality is defined as belonging simultaneously to multiple distinct social groups, each of which can experience discrimination (Crenshaw, 1991). The concept was originally articulated in relation to discrimination faced by Black women, which overlapped with, yet was distinct from, that faced both by other women and by Black men. Crenshaw (1991) noted that discrimination is imposed in different ways depending on the social groups to which someone belongs, and intersectionality thus

TABLE 1 Description of Reviewed Studies						
Authors and Year	Study Purpose	Country	Study Designs	Methods	Participants	Summary of Findings
Granerud & Toft (2015) 1	Individual perspectives on opioid rehabilitation with BUP and MET	Norway	Exploratory grounded theory approach	Semistructured interviews. Each interview, new questions were introduced for rich perspective.	<i>n</i> = 7 Norwegians from OMT recruited through local charity and Facebook, ages 26–49 years, female (<i>n</i> = 4) and male (<i>n</i> = 3)	“A better life if you follow the rules of the game.” Objectification and disenfranchised in OMT.
Grønnestad & Sagvaag (2016) 2	Individual experiences of OMT and the impact on recovery	Norway	Ethnography	Researcher spent time in drug scene for a 1-year span. Field notes and audiotaped interviews.	<i>n</i> = 21 Norwegians from the OMT program and the open drug scene. Average age of 40 years, female (<i>n</i> = 3) and male (<i>n</i> = 18).	OMT overruling and degrading. Does not remove painful emotions. Experienced stigma and loss of agency.
Hatcher et al. (2018) 3	Examines differences in BUP experiences by race, ethnicity, and socioeconomic status	United States	Exploratory grounded theory approach	Open-ended qualitative interview questions about participants' perceptions of stigma and the social context.	<i>n</i> = 77 receiving BUP in a primary care clinic and two OTPs. Average age of 46 years, female (<i>n</i> = 17) and male (<i>n</i> = 60). Race, ethnicity, education, and housing reported.	Management of stigma and perception of social needs varied significantly by ethnicity, race, and SES. The study found structural barriers and structural violence in treatment.
Hewell et al. (2017) 4	Examines attitudes, perceptions, and knowledge of MOUD	United States	Exploratory qualitative grounded theory approach	Analyzed data from two focus groups and three in-depth interviews with pts who use BUP or considering using BUP for treatment	<i>n</i> = 11 Alaskan patients dx OUD and, at some point, had been impacted by MOUD. Age ≥ 18 years, female (<i>n</i> = 7) and male (<i>n</i> = 4).	Conceptual model revealed individual and systemic factors as well as domains that influence the lived experience.
Larney et al. (2017) 5	Patient reasons for seeking and ceasing MOUD in prisons in New South Wales	Australia	Qualitative descriptive	Semistructured interviews, 30- to 60-minute interviews	<i>n</i> = 46, seven correctional centers in Sydney. Average age of 35 years, female (<i>n</i> = 14) and male (<i>n</i> = 32). Ethnicity, tx type, education, and time in custody.	Facilitators for seeking MOUD and reasons for ceasing included concerns related to return to drug use, inconvenience, opposition, and stigma.

(continues)

TABLE 1 Description of Reviewed Studies, Continued						
Authors and Year	Study Purpose	Country	Study Designs	Methods	Participants	Summary of Findings
Lindgren et al. (2015) 6	Lived experiences of individuals participating in MOUD with OUD	Sweden	Qualitative, descriptive, phenomenological hermeneutic interpretations	Narrative interviews, 15- to 47-minute interviews (average = 34 minutes)	<i>n</i> = 11, psychiatric clinic, MOUD tx length ≥ 3 years (median = 7 years) Age range = 27–51 years, average age = 37 years, female (<i>n</i> = 5) and male (<i>n</i> = 6)	Major theme: “resistance to existence” The participants seized the chance to claim a life lived with dignity, struggled with hidden challenges, and eventually were freed from their pasts.
Redden et al. (2013) 7	Metaphor analysis as a linguistic tool to understand the cultural experiences of using MOUD	United States	Qualitative descriptive/grounded analysis	Focus groups with an interview guide to frame discussion during 90-minute sessions with a 10-minute break	<i>n</i> = 68, six metro areas in MOUD clinics from ethnic minority groups, MOUD tx length ≥ 4 months Ages 22–82 years Education and employment noted.	Methadone was described as “liquid handcuffs” that allowed those in recovery to quit and get “straight.” Methadone was also seen as a “crutch.” Loss of agency with MOUD. Drug use: active vs. recovery passive.
Vishwanath et al. (2019) 8	Describe the experiences of opioid replacement in rural and regional areas	Australia	Qualitative, interpretative phenomenological approach	Narrative interviews, 25–45 minutes in length	<i>n</i> = 16, five townships, ORT program, MOUD tx length ≥ 5 years, ages 30–55 years. Female (<i>n</i> = 4) and male (<i>n</i> = 12). Education and employment noted.	Found psychosocial impacts on ORT relating to financial and social burdens, stigma, and discrimination.
Wilson et al. (2018) 9	Examines the experience when individuals first initiate the use of opioid medicine to treat pain through enrollment in an outpatient MOUD program	United States	Qualitative grounded theory approach	Face-to-face, semistructured interviews, 45–90 minutes in length	<i>n</i> = 10 pain self-management program/outpatient MOUD. Average age of 47.6 years, female (<i>n</i> = 4) and male (<i>n</i> = 6). Education, employment, marital status, race/ethnicity, and pain syndrome noted.	Theory creation, living with persistent pain. Opioid initiation for substance use treatment. Three emerging categories: (a) addiction pathway, (b) relationship spectrum, and (c) becoming normal.

BUP = buprenorphine; MET = methadone; OUD = opioid use disorder; MOUD = medication for opioid use disorder; MOT = Opioid maintenance treatment; ORTs = Opioid Treatment Programs; SES = Socioeconomic Status; pts = Patients; dx = Diagnosis; tx = Treatment; ORT = Opioid Replacement Therapy.

TABLE 2 Derived Themes			
Derived Analytic Theme and Subthemes	In Paper # (as Listed in Table 1)	Primary Study Themes (as Labeled)	Supporting/Essential Quotes
1. The duplexity of MOUD creates ambivalence in how a person experiences MOUD			
Savior versus oppressor	Savior: 2, 4, 5 (p. 307), 6, 7 (p. 956), 8	Savior: get your life back, miracle, lifesaving, prevents disease, MOUD as lifeline, saved from self-destructive tendencies, savior, safety net, regained ability to cope with pain	"But then, from having started to feel better, from thinking about suicide as a form of relief, to actually wanting to live to no longer thinking about killing myself, as I had done every day before...for I don't know how many years. I no longer think like that, so it's like going from a dark autumn storm to a sunny summer's day...it's absolutely enormous" (6, p. 966). "It's better to be on methadone than have hep C" (5, p. 307). Equated his addiction to "quicksand that nearly suffocated" him; he conceptualized methadone, however, as a savior that allowed his loved ones to "get bits and pieces of me back" (7, p. 956).
	Oppressor: 1, 2 (p. 5), 3, 4 (p. 6), 6, 7	Oppressor: fear sanctions, reprimand by principal, surveillance and intersectionality of multiple oppressions, restriction of treatment, controlling elements, liquid hand cuffs, acts as crutch	"So, my attitude has always been: 'Say as little as possible, and certainly nothing that can be used against you later on'" (2, p. 5). "Umbilical cord" (4, p. 6).
Agency versus powerlessness	Agency: 2 (p. 4) 3, 4, 5, 7 (p. 955), 8	Agency: increased self-confidence leads to better job, advocating for oneself, strong enough to get through stigma, gaining status through employment that equates to autonomy, perceiving choice to make changes, choice to stay on or off before release from jail, self-efficacy and personal control, actively took steps to find help for their addiction	"There is help out there. It depends on you. You have to go get it. It's not going to go to your house. The same way you go get the drugs is the same way you go get the help" (7, p. 955). "And then I got the necessary self-confidence and I dared to go out...and now I actually apply for proper jobs" (2, p. 4).
	Powerlessness: 2 (p. 5), 3, 7 (p. 957)	Powerlessness: asymmetrical power balance, lack of codetermination, inability to control and manage information regarding addiction, forced detox due to powers outside the person's control, feeling powerless while weaning off like a newborn, poor treatment by staff, uneducated providers	"You have no influence over the treatment" (2, p. 5). "If you lose your Medicaid card, they are going to detox you so fast that you gonna spin right out the door and start using drugs" (7, p. 957). "Newborn baby" (7, p. 57).

(continues)

TABLE 2 Derived Themes, Continued

Derived Analytic Theme and Subthemes	In Paper # (as Listed in Table 1)	Primary Study Themes (as Labeled)	Supporting/Essential Quotes
Normalizing versus stigmatizing	Normalizing: 2, 6 (p. 967), 8 (p. 374)	Normalizing: better human being, regain everyday life, feel like normal human beings, return of dignity, becoming normal	"It's important to feel like a normal human being" (6, p. 968). "Normal life" (8, p. 374).
	Stigmatizing: 1, 2 (p. 6), 3 (p. 305), 4, 5, 6, 7, 8 (p. 375)	Stigmatizing	"You feel like a criminal. It is as if they assume that you will try to trick and cheat. As if they think that after all you are just a junkie, so they cannot entirely trust you" (2, p. 6). "And then you get your help somewhere else, counseling and NA meetings. I don't mention Suboxone because more than half of the people there will give you a hard time 'you're not really clean!' I just don't mention it. I don't bother or get into it or talk about it" (3, p. 305). "So, you know, it makes you scared to even say anything about your problems...and the first thing he told me was, 'Don't expect to get anything from me,' pretty much. Which is wrong. I mean, that's totally wrong... I can't get past that to—to address all of my pain issues" (8, p. 375).
		<i>Internal</i> (self-stigmatizing): shame, inferiority <i>External</i> (professionals, law, family, public, recovery community): seen as crutch, suspicion, accused of hidden agendas, you're not really clean, overt and covert stigma, social stigma stymie's choice, replacing one addiction with another, double stigma (OUD + MOUD), moral failing and MOUD as crutch or shortcut—something to lean on when they could be independently strengthening themselves, MOUD as a medication or a drug, addiction versus chronic health condition, judged by health care providers in their recovery from addiction and pain, accusatory looks	
Abundance versus sacrifice	Abundance: 2, 5, 6 (p. 968), 7 (p. 956)	Abundance: satisfaction with life, avoiding dealer debt, regain pleasure of life, peaceful life, return of dignity, wholeness, finances improved, money in pocket, good life—meaning housing, hygiene, freedom, transportation, family, relationships	"Eating better, not just noodles or going without" (6, p. 968). "The good life" (7, p. 956).
	Sacrifice: 1, 2 (p. 4), 3, 4, 6	Sacrifice: balancing demands of treatment with other responsibilities, freedom with limits, cost of treatment, having to travel long distances in dangerous circumstances, committing a crime to get treatment, making sacrifices—breaking contact with old friends, having a small life to stay out of trouble	"For me who wants to become totally drug free it is positive that with OMT you have a certain supervision. That there is a certain control and meetings with the responsibility group" (2, p. 4).
Connection versus isolation	Connection: 6, 8	Connection: repair relationships, functioning social network, making amends, reconnection with family	
	Isolation: 5 (p. p.308), 7 (p. 958).	Isolation: from old friends, cloistering to staying out of trouble and avoid bad influences	"I get on the bus, and I go straight home, and work and I stay inside. You know what I mean? I haven't been back to prison in going on six years" (7, p. 958).

(continues)

TABLE 2 Derived Themes, Continued

Derived Analytic Theme and Subthemes	In Paper # (as Listed in Table 1)	Primary Study Themes (as Labeled)	Supporting/Essential Quotes
Biological blessing versus curse	Blessing: 2 (p. 4), 3, 4, 5, 6, 8	Blessing: manages cravings, addresses cravings, manages withdrawal and cravings, prevents cravings and withdrawal, decreases anxiety, improves sleep, helps withdrawal and cravings	"Before it was the first thing you thought about when you woke up in the morning and the last thing you thought about before you went to bed, and the whole day centered around that. But now you wake up in the morning, put a pill under your tongue and get going, and the day is just like any other day. You don't think about it 24 h a day, like you did before" (2, p. 4).
	Curse: 5, 6 (p. 965), 8	Curse: medication side effects; constipation, indigestion weight gain, dulling of emotions, difficult withdrawal from MOUD	"The medicine puts a lid on life, emotions, and quick-wittedness" (6, p. 965).
2. The structure of treatment and systemic influences matter			
Medication choice makes a difference in experience: buprenorphine (BUP) vs. methadone (MET).	3 (p. 305), 4, 5 (p. 308)	BUP: life without fear of exposure, BUP delivery in primary care setting enables treatment without punishment, surveillance, and/or collateral consequences, pass as nonaddict, flexibility with BUP, can hold a job, BMT equals gaining status, freedom and flexibility that fosters strength Less stigma attached to BUP	"I just wouldn't want my family knowing [about being on methadone]. 'Bup, yeah.' Interviewer: 'So what's the difference between your family knowing you're on methadone versus Bup?' Participant: 'Well, they know what methadone is for. Bup, you can tell them it's for an injury or something'" (5, p. 308).
	2, 3 (p. 305), 5 (p. 308), 7	MET: delivery in opioid treatment program (OTP), greater control measures, surveillance, meetings never ended, demotivating structure of OTP, framework left people feeling powerless, MET reminds patients of their addiction, greater stigma with MET, feeling like a criminal, separated from general population, ties a person to crime, depersonalization, long-term commitment, daily obligation impacts schedule, bound to clinic, more difficult withdrawal	"[Taking] methadone every day, and going to treatment means seeing people who constantly remind me of my addiction, ...meetings, and methadone treatment are a belt like factory and in a group with people with very little in common" (3, p. 305). "Avoid old crowd "It [the methadone clinic] ties me in with crime, with the wrong people" (5, p. 308). "It's on my menu now. Seven days a week. I don't like that" (7, p. 958). "I'm not exactly free. I still have to plan around the hours of the clinic, and it is not as easy to get 'take homes' here" (7, p. 958).

(continues)

TABLE 2 Derived Themes, Continued

Derived Analytic Theme and Subthemes	In Paper # (as Listed in Table 1)	Primary Study Themes (as Labeled)	Supporting/Essential Quotes
Intersectionality of multiple oppressions: inequalities of race, class, law enforcement, housing, and education	2, 3, 4 (p. 5), 5, 6, 7 (p. 957)	Structural violence, oppressive surveillance, predisposes pt. to violence, lack of education impacts pt. advocacy, social control/funding of treatment, social stratification, housing dependent on treatment, institutional neglect, cultural context impacts societal stigma, mandated services incriminate pt. and cause trauma, fear of arrest and losing access to MOUD in jail, war on resources, financial hardships	“I think being Alaska Native and having addiction and trying to get help, there's a huge stigma to that. It's kind of like, you're a shame. It's like very shameful. We're very prideful people, you know. It's a lot harder for people to get clean” (4, p. 5). “If you lose your Medicaid card, they are going to detox you so fast that you gonna spin right out the door and start using drugs” (7, p. 957).
3. MOUD as a stepping-stone with multiple meanings in recovery			
Harm reduction approach	1, 2 (p. 5), 4, 8	MOUD achieves harm reduction—reducing or abstaining from illicit opioids even if using other substances, harm reduction approach preferred over abstinence model by participants. Recognizes progress, acceptance of transgressions, and unconditional support and positive regard.	Clean urine samples are all that is important: “That's a little special with OMT, they are very drug focused. It is not so important that you have bought an apartment or go to school or it's like ‘Yeah, but have you been taking drugs?’ You can sit all alone at home and have anxiety and a terrible life, but clean urine samples. And then you are sort of clever!” (2, p. 5).
More tools in the arsenal MOUD and psychosocial treatments in combination	1 (p. 255), 6 (p. 967), 7 (p. 958), 8	Psychosocial treatments helpful, learning healthy coping skills, thinking clear, need to talk and address trauma, tools in arsenal, counseling important to teach coping skills	“The counselors teach you how to think clear because when you're using drugs, you aren't thinking straight. Your mind changes completely, you know, and it gets your mind thinking, I can start life again” (1, p. 255). “A lot of people with addiction have been through quite a lot of traumas, so you might consider dialog. Some people have a very urgent need to talk” (6, p. 967). “Tools” in their arsenal—support groups, social support—as ways to manage anxiety and survive the potential ‘war on resources’” (7, p. 958).
MOUD as a stepping-stone in the recovery process	4 (pp. 4, 7), 6, 7 (p. 958), 8	Stepping-stone toward recovery and going through a process of change, living a program of recovery, not a permanent point, different for every person, continuous healing and gradual improvement, MOUD to prevent relapse and falling back, getting on MOUD was a turning point.	“I can't stop. It never stops, the disease doesn't go away” (4, p.4). Not a “quick fix” (4, p. 7). Avoid being “dope sick” going through withdrawal and “slipping back into addiction” (7, p. 958).

MOUD = medication for opioid use disorder.

epitomizes the complexities and systemic nature of social determinants and is the antithesis of atomization.

The primary themes guiding this new synthesized understanding are as follows: (a) The duality of MOUD creates ambivalence in how a person experiences MOUD, (b) the structure of treatment and systemic influences matter, and (c) MOUD as a stepping-stone with multiple meanings. Through reciprocal translation, primary studies are numbered and connected to these themes (see Table 1) in the sections below.

Theme 1: The Duality of MOUD Creates Ambivalence in the How a Person Experiences MOUD

Fundamental dichotomies emerged in how individuals with OUD or dependency experienced MOUD, which creates ambivalence in perception and choice. This fundamental dichotomy can be conceptualized through the Chinese concepts of yin and yang, which represent two opposing, yet inseparable, forces attempting to gain dominance over each other. Although one force can briefly become ascendant, it cannot exist without the other, and this conflict forms the basis of both real and experiential dualities. The theory of holistic duality postulates that “a phenomenon or entity cannot be complete unless it has two opposite elements” (Fang, 2012, p. 26), which can be further illustrated by the social theory of DuBois who posited, in 1904, a hyphenated reality for African Americans in terms of double consciousness:

...the Negro...is born with a veil and gifted with second sight in this American world—a world which yields him no true self-consciousness.... It is a peculiar sensation, this double consciousness, this sense of always looking at oneself through the eyes of others, of measuring one's soul by the tape of a world that looks on in amused contempt and pity. One ever feels his two-ness—an American, a Negro; ...two unreconciled strivings; two warring ideals in one dark body, whose dogged strength alone keeps it from being torn asunder. (Du Bois, 1903, p. 11)

In his article about double consciousness and the “spirit” of recognition, Meer (2019), claimed that DuBois summoned the dialectic of lived experience for unrecognized, marginalized identities and their transformative potential. Although yin/yang creates context for holistic duality, the importance of DuBois' work calls for social change within the lived experience particular to people experiencing OUD, the stigma that exists for them, and the treatment that follows.

The data yielded eight subthemes of experiential duality: (a) savior/oppressor, (b) agency/powerlessness, (c) normalizing/stigmatizing, (d) abundance/sacrifice, (e) connection/isolation, and (f) biological blessing/biological curse, as shown in Figure 2. The eight subthemes seem to encompass elements of social justice, and the apparently diametrically opposed extremes expose a system at odds with itself—two unreconciled aspirations and two conflicting beliefs. The first subthemes show

that MOUD was viewed as a “savior” (Redden et al., 2013) and a way for individuals to reclaim their lives, but it was also viewed as a system of oppression with an asymmetrical power balance and surveillance that degrades hope. One participant felt oppressed because they were required to report daily to the MET clinic, remarking that the clinic was an “umbilical cord” (Hewell et al., 2017, p. 6).

MOUD also increased individual agency and self-efficacy in choice and in personal control over OUD; it was a way of returning to a normal life in which dignity was restored, status was gained, and confidence grew in recovery. One participant said, “I got the necessary self-confidence, and I dared to go out, and I actually apply for proper jobs” (Granerud & Toft, 2015, p. 4). Conversely, participants felt powerless because of the lack of co-determination in deciding their treatment or of the ability to manage personal information about their OUD. They feared that they would be detoxed against their will or that the asymmetrical power balance within the MOUD treatment structure would leave them feeling defenseless. One patient stated, “If you lose your Medicaid card, they are going to detox you so fast that you gonna spin right out the door and start using drugs” (Redden et al., 2013, p. 957).

Furthermore, participants viewed MOUD as both normalizing and stigmatizing; being administered MOUD meant they could pass as a “nonaddict” and “seize a normal life with dignity” (Lindgren et al., 2015, p. 965). For some participants, MOUD also provided a routine that normalized daily living. Notably, the theme of stigmatization was found in all nine studies: They highlighted both external stigma and the subsequently internalized oppression that the participants faced. Participants expressed shame and inferiority, and they were stigmatized by law officials, family members, medical providers, the public, and individuals within the recovery community. A common stigmatizing belief with which they struggled was the idea that MOUD substituted one drug for another. As one participant stated, “You're not really clean!” (Hatcher et al., 2018, p. 305).

Abundance versus sacrifice also surfaced as a theme. For many individuals, “getting their lives back” represented abundance in terms of regaining pleasure in life, improving finances, restoring family connections, eating better, obtaining housing, and gaining freedom through transportation. One participant referred to it as “the good life” (Redden et al., 2013, p. 956). However, MOUD came with sacrifices, such as treatment center demands, the cost of treatment, traveling long distances, braving unsafe situations, and not being able to travel or spend time with family. In addition, participants recognized that they had to sacrifice elements of personal control to stay in recovery and noted experiences of connection and isolation. Connection was viewed as “the good life” (Redden et al., 2013, p. 956); MOUD helped participants repair relationships and reconnect with families and social networks. Conversely, MOUD encouraged participants to stay out of trouble and thus isolate themselves from the bad influence of some old friends; as one individual recounted, “I get

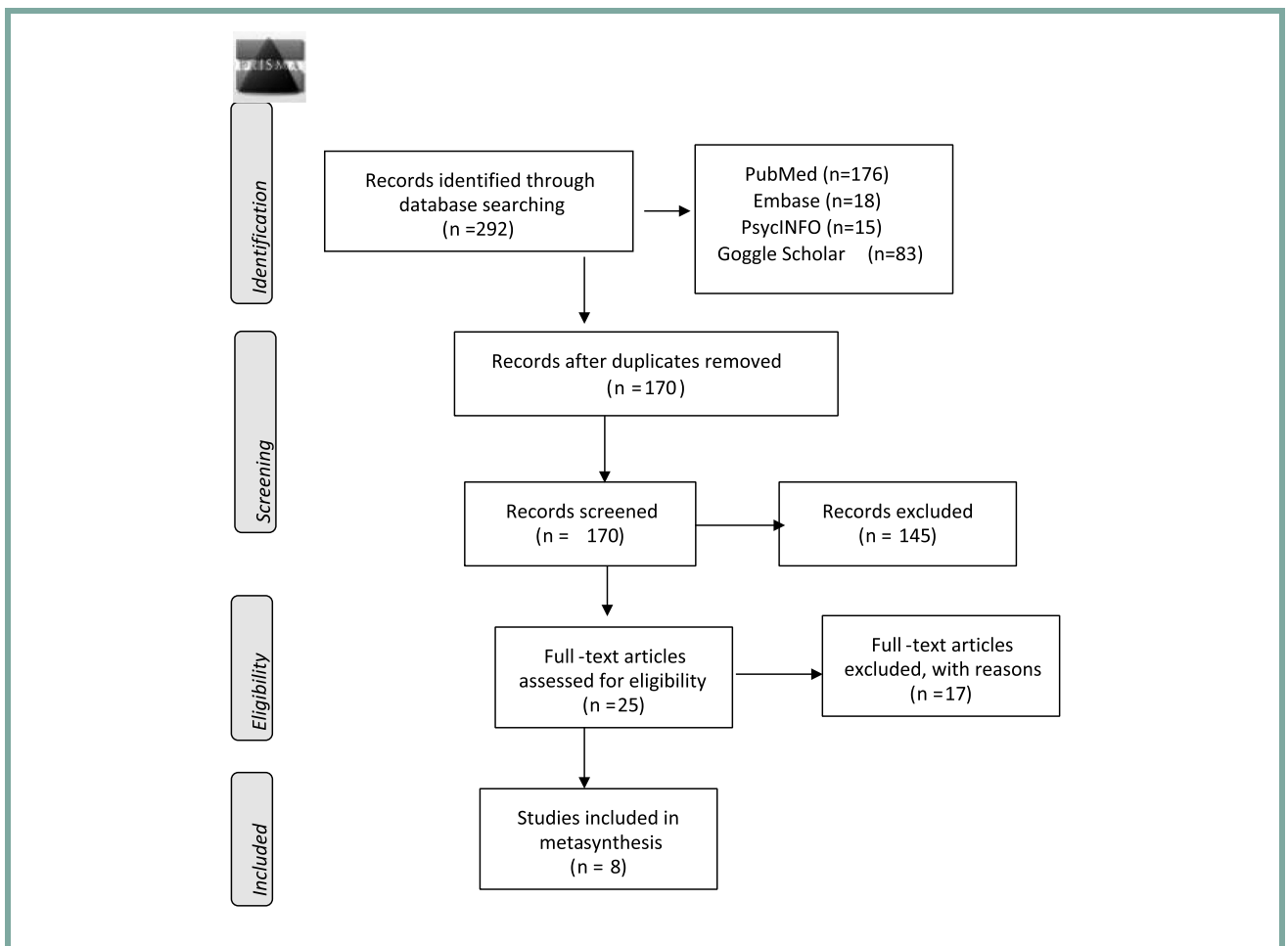


Figure 2. Medication for opioid use disorder experienced as an ambivalent experience influenced by intersectionality. BUP = buprenorphine; MET = methadone.

on the bus, and I go straight home, and work and I stay inside” (Redden et al., 2013, p. 958).

Finally, participants viewed MOUD as both a biological blessing and a curse. On one hand, the medication helped many participants manage cravings and withdrawal and to improve sleep and anxiety. On the other hand, it caused its own intolerable side effects, such as dulled emotions, which made it difficult to experience withdrawal. One participant lamented, “The medicine puts a lid on life, emotions, and quick-wittedness” (Lindgren et al., 2015, p. 965).

Theme 2: The Structure of Treatment and Systemic Influences Matter

This theme elucidates how elements of identity and intersectionality influence medication choice (BUP vs. MET) in the lived experience of MOUD participants. BUP was viewed as a more flexible choice, fostering strength, and offering a life without fear of exposure (Hatcher et al., 2018) because it is prescribed in a primary care setting with less surveillance and more anonymity. As a result, participants perceived BUP as less stigmatizing: “I just would not want my family knowing [about being on methadone]. BUP, yeah” (Larney et al.,

2017, p. 305). MET, however, was perceived as having greater stigma, surveillance, and obligations in a treatment structure that felt oppressive, demotivating, and depersonalizing. Participants also worried about destructive and criminal behaviors—“[The methadone clinic] ties me with crime, with the wrong people” (Larney et al., 2017, p. 308)—but opioid treatment program protocols that were more relaxed might lead to MET diversion and individuals worrying that they could fall victim to destructive behaviors and patterns. Finally, the lived experience of MOUD participants is defined by intersecting oppressive inequalities, which are connected to marginalized identity markers, including race, class, housing, and education (Hatcher et al., 2018). For example, educated White women could secure BUP more easily, providing evidence of systemic racial identity privilege. In this context, White MOUD participants are protected by the color of their skin; they can pass as “nonaddicts” and receive better care (Hatcher et al., 2018). Furthermore, lack of education and social status negatively impacts self-advocacy, and the “unidimensional nature of pharmacotherapy exacerbates underlying inequities in housing and social support” (Hatcher et al., 2018, p. 306).

Notably, participants undergoing mandated treatment were isolated from their peers and subjected to oppressive surveillance, which further stigmatized and incriminated them, leading to trauma and structural violence. Social control and unstable funding were also perceived as a “war on resources” in which participants needed “armor” (Redden et al., 2013, p. 958) for the battle.

Theme 3: MOUD as a Stepping-Stone With Multiple Meanings in Recovery

Finally, the perception of MOUD in the context of recovery revealed three subthemes: (a) MOUD as a method of harm reduction, (b) combining MOUD and psychosocial treatments, and (c) MOUD as a transition stage in the recovery process. Overall, participants viewed MOUD as a harm reduction strategy to reduce or curtail illicit use. How the provider viewed harm reduction made a difference in how the individual experienced MOUD in recovery—it played either a demotivational or supportive role. For example, focusing on abstinence and negative urine samples while excluding other successes was demoralizing (Granerud & Toft, 2015), whereas accepting transgressions as a part of the recovery process was viewed as being supportive (Wilson et al., 2018).

Participants believed that medication alone was insufficient and that more tools were needed to address OUD effectively. Psychosocial treatments in combination with MOUD were favored by most participants to address trauma, learn coping skills, and deal with the “war on resources” (Redden et al., 2013, p. 958). One participant stated that “a lot of people with addiction have been through quite a lot of traumas, so you might consider dialog. Some people have a very urgent need to talk” (Lindgren et al., 2015, p. 967).

Finally, most participants viewed MOUD as a stepping-stone and not a quick fix on the road to recovery (Hewell et al., 2017). This subtheme speaks to a lifelong process that is different for every person (see Figure 2). All three subthemes promote a shift toward a more diverse, holistic, and biosocial approach to treatment that considers the participant within their social location or lived environment.

DISCUSSION

Theoretical Orientation

As stated previously, participants' experiences with MOUD have traditionally been measured using quantitative techniques that favor atomized, rational data over biosocial, systemic data that are situated in a person's social location or lived experience and intersecting identities. Farmer et al. (2006) recognized that, with the historically atomistic nature of science, medical and addiction professionals are not conditioned to address structures or track systemic dynamics. The current study highlights the importance of qualitative research in understanding the lived experience of MOUD and the participants' biosocial constructs. It also highlights the need for a more postmodern theoretical approach—specifically feminist standpoint epistemology and DuBoisian social theory—to develop more effective

OUD and substance use disorder (SUD) treatment strategies.

The Problem With Individualism

Individualism—as a construct in American society—is the notion of pulling oneself up by one's bootstraps and being a “self-made man.” This rugged individualism attributes successes and failures to the strengths and weaknesses of one's character rather than systemic social structures. Focusing on the individual implies that group memberships, one's social location, and intersecting identities are not important or relevant to opportunities (DiAngelo & Sensoy, 2014). Farmer (2006) noted that “discussions of substance abuse are curiously dissocialized, viewed as personal and psychological problems rather than societal ones. Here, too, structural violence is perpetuated through analytic omission” (p. 1690) and atomizes those using substances to their individual experiences and/or decisions. This metasynthesis highlights the economic, biological, social, and environmental imbalances directly attributable to the distorted equilibrium that fosters ambivalence in the perception of MOUD. It begs a wider lens and a shift from the individual to an understanding of systemic, structural, and oppressive dynamics, and it necessitates the examination of the social determinants of health (SDoHs) in medicine and public health.

The human condition is driven by a biological directive to move toward pleasure and away from pain. This tenet takes on an exquisitely important meaning in the context of OUD because hypersensitivity to physical and emotional distress is a consequence of opioid use, rendering an individual less tolerant of paradoxical experiences, especially when they are subjected to negative states such as oppression, powerlessness, stigmatization, sacrifice, isolation, and intolerable medication side effects, in addition to withdrawal. There is therefore a need to validate and normalize duplexity and to show that individuals with OUD are gifted with a special second sight.

Double Consciousness, Second Sight, and the Problem

W. E. B. DuBois fleshed out the inherent duality for Black people in America, a double consciousness that has application in the lived experience of those with OUD. Persons experiencing OUD exist as both those who are in recovery and those who have used substances, a layered duality seen throughout this study. This double consciousness provides a normative quality for those experiencing OUD in that it captures the duality of voices going unrecognized and highlights their transformative potential (Meer, 2019). It acknowledges the way in which those voices are prevented from effecting change for themselves and others because of their marginalization, which is based on opioid use.

There is consequently a gift of a second sight born out of the double consciousness, which has been postulated by theorists like DuBois and Karl Marx as well as feminist social epistemologists, the latter of whom argue that those with marginalized identities have epistemic privilege—their “social locations offer the potential to be more epistemically reliable than others”

(Grasswick, 2006, p. 2). Marx's social theory of the marginalized working class and privileged capitalists shows that the working class has a strong motivation to acquire knowledge of social locations to navigate the oppressive system; conversely, capitalists are motivated to understand class structures only so that they can reinforce their privileged positions (Grasswick, 2006). The working class therefore has a second sight, although both the working class and the capitalists are driven by survival—one to sustain the status quo and the other to disrupt it.

The second sight granted by the lived experiences of those participating in MOUD reflects epistemic privilege and a more knowledgeable understanding of the world. Families, society, and treatment providers have often conveyed, explicitly or implicitly, that those struggling with addiction are the problem, inducing feelings of guilt and shame; imagine if they were told that, rather than being the problem, they are gifted with second sight.

This disequilibrium is often less noticeable with BUP than with MET. There is less intersectionality imposed on treatment using BUP, making it a more tolerable and flexible option that fosters strength (see Figure 2; Hewell et al., 2017). BUP also provides less exposure and more anonymity. In contrast, MET treatment is associated with greater oppression and structural violence. A new ideological approach to SUD treatment also values and prioritizes intersectionality, SDoH, and systemic, biosocial therapeutic interventions, thus avoiding reducing people to simply good or bad decisions.

NEW TREATMENT APPROACH

A more effective approach to treatment starts with the theoretical shift from individualism and atomism, in which participants are reduced to characterizations of personal strengths/weaknesses and individual choices, to a systemic and biosocial approach, which considers and values intersecting identities and celebrates the duality or second sight of participants. Providers should not dissuade participants from finding value in either side of their inherent duality—their recovery versus substance use.

Those undergoing treatment for SUD should not define themselves as addicts or demonize themselves for struggling with substance use because both versions of themselves—the self in recovery and the self struggling with substance use—are essential and provide opportunities for learning, healing, and growth (Du Bois, 1903). Attempting to satisfy or eliminate the tension between the two unreconciled sides wreaks havoc on the courage of the individual and, at times, leads to a sense of shame—being ashamed of oneself and of the past (Du Bois, 1903). Current treatment approaches and societal perspectives often encourage those who use substances to be ashamed of themselves, which contributes to low self-worth and damages self-efficacy. Being satisfied with something lesser can only amount to so much.

Ultimately, providers must listen to individuals undergoing treatment for SUDs. If providers value the epistemic privilege of the lived experience, the power dynamic between provider and participant could shift, uniquely empowering the participant—this is how to truly honor the lived experience and how providers

can help participants to maintain a sense of self in response to misrecognition (Meer, 2019). Moreover, MOUD is only one approach to recovery. Efficacious treatment requires the incorporation of psychosocial interventions and the appreciation of biosocial dynamics, which have largely been reserved for the treatment and prevention of “social illnesses” like HIV, asthma, and lead poisoning (Farmer et al., 2006). There must be wider application of psychosocial and biosocial dynamics such that any illness may be considered a social illness; harm reduction should be a treatment focus—valuing improved social functioning over abstinence—and an acceptable, patient-centered goal.

Preliminarily, our metasynthesis uncovered significant gaps in the literature and additional areas for research work, such as an appreciation of the differences between BUP and MET in MOUD programs and a focus on evaluating social justice frameworks to further elucidate aspects of intersectionality with MOUD participants, which could enhance apperception and improve treatment delivery. An understanding of experiential dualities allows for a diversity of perspectives, informs interventions and treatment structures, and can serve as a backdrop for social change, and professionals should recognize the double consciousness—the paradox of the MOUD experience—and work to value and validate the inherent dichotomies. This means reevaluating mandated treatment and the surveillance levels associated with MOUD and allowing the individual to have a greater say in treatment to increase agency, which entails listening closely to concerns related to medication side effects and the limitations associated with MOUD. Professionals can also advocate for programs such as MOUD Anonymous (12-step facilitation) that destigmatize MOUD and can decrease isolation. Education can also be provided to the public and to families about the false narrative of substituting one drug for another drug.

To balance the sacrifice felt in the MOUD experience, a professional can institute low-barrier approaches with BUP treatment that emphasize convenience and flexibility. Telehealth, same-day initiation, home inductions, and microdosing treatment strategies are all examples of low-barrier approaches that improve retention in MOUD programs and increase access to care. Outpatient providers have observed improved treatment engagement with telehealth throughout the COVID-19 pandemic, including a 50% drop in no-show rates for new-patient intakes (Clark et al., 2021). Many socioeconomic and geographic treatment hurdles that now limit access to BUP are bypassed with telephone and Internet assessments and should be allowed to continue postpandemic (Clark et al., 2021). This would require several regulatory and legal changes, including Congress amending the Ryan Haight Act to allow waived providers to initiate BUP without an initial in-person visit (Clark et al., 2021). Future research is also needed to determine whether telehealth can effectively address barriers to care and promote engagement and whether the privacy provided by not visiting a clinic makes patients feel less stigmatized (Clark et al., 2021).

The last element to reducing ambivalence is to employ techniques such as motivational interviewing to understand the personal narratives of MOUD and the individual

motivations for change and to honor and respect the individual's "second sight" to reduce shame and promote self-efficacy. To address the intersectionality of the MOUD experience, professionals should seek to achieve health equity and should advocate for social change. Professionals can also work with government agencies to ensure social funding to support the cost of MOUD. On the clinical side, professionals can screen for SDoHs and develop a plan of action to address them. They can also seek to enact culturally and linguistically appropriate health services and health care guidelines to standardize care as a way of addressing health disparities.

The health care practice of broaching could also be employed; this is a tool to reduce power imbalances, promote trust, and reinforce therapeutic partnerships (Day-Vines et al., 2007) and refers to a counselor's direct attempts to initiate and respond to sociocultural and sociopolitical factors during treatment. Broaching includes discussing awareness of race, ethnicity, culture, and other obvious differences (such as White privilege) to establish connections and encourage open dialog about diversity.

LIMITATIONS

Some of the limitations of our research are related to the scope, quantity, and quality of the qualitative studies included in the analysis. The treatment settings are another limitation—studies from outside the United States were evaluated, as well as outpatient and opioid treatment programs that represent different systems of care. Despite these limitations, this study has several strengths, including the reciprocal translation process, which hinges on the diverse knowledge and background of the team members to bring unique perspectives on MOUD.

CONCLUSION

In summary, MOUD is an ambivalent experience influenced by intersectionality, and these findings shed light on what is missing and helps to inform the question of why this form of treatment remains vastly underutilized and controversial. Metasynthesis produced three overarching themes: (a) The duality of MOUD creates ambivalence in how a person experiences MOUD, (b) the structure of treatment and systemic influences matter, and (c) MOUD as a stepping-stone with multiple meanings in recovery. The findings of this metasynthesis evoke discussions of philosophical constructs and an application of DuBoisian social theory that is rooted in the lived experience of MOUD participants, thus inspiring a strengths-based, person-centered approach to SUD treatment.

Understanding experiential dualities, appreciating identity and intersectionality, and realizing where these lie in the conversation about recovery allow for a more holistic and multifaceted conceptualization of the lived experience that will increase the provider's ability to see different perspectives and create space for the diversification of interventions and treatment structures. The opportunity exists for a shift in the power imbalance in the provider/client relationship, for validating and balancing the paradoxes and the yin and yang of the MOUD experience, for

linking experiences to biosocial constructs, and for reducing the likelihood of enacting structural violence, all of which make possible improved engagement with treatment and the undoing of this devastating crisis.

REFERENCES

- Agamben, G. (1998). *Homo sacer: Sovereign power and bare life*. Stanford University Press.
- American Medical Association. (2021). Issue brief: Nation's drug-related overdose and death epidemic continues to worsen.
- American Psychiatric Association. (2013). *Diagnostic and statistical manual of mental disorders (DSM-5)* (5th ed.). American Psychiatric Publications.
- American Society of Addiction Medicine. (2016). *Opioid addiction 2016 facts & figures*. https://www.asam.org/docs/default-source/advocacy/opioid-addiction-disease-facts-figures.pdf?sfvrsn=100c76c2_12?search=%22facts%20figures%22
- Anderson, E. (2020). Feminist epistemology and philosophy of science. In Zalta, E. N. (Ed.), *The Stanford Encyclopedia of Philosophy (Spring Edition)*. <https://plato.stanford.edu/archives/spr2020/entries/feminism-epistemology/>
- Bronfenbrenner, U. (1992). *Ecological systems theory*. Jessica Kingsley Publishers.
- Case, A., & Deaton, A. (2017). Mortality and morbidity in the 21st century. *Brookings Papers on Economic Activity*, 2017(1), 397–476. <https://www.nber.org/mortalityand-morbidity-in-the-21st-century/casetextsp17bpea.pdf>
- Centers for Disease Control and Prevention. (2017). Opioid overdose. <https://www.cdc.gov/drugoverdose/index.html>
- Clark, S. A., Davis, C., Wightman, R. S., Wunsch, C., Keeler, L. A. J., Reddy, N., & Samuels, E. A. (2021). Using telehealth to improve buprenorphine access during and after COVID-19: A rapid response initiative in Rhode Island. *Journal of Substance Abuse Treatment*, 124, 108283.
- Connery, H. S. (2015). Medication-assisted treatment of opioid use disorder: Review of the evidence and future directions. *Harvard Review of Psychiatry*, 23, 63–75. 10.1097/HRP.0000000000000075
- Crenshaw, K. (1991). Mapping the margins: Intersectionality, identity politics, and violence against women of color. *Stanford Law Review*, 43(6), 1241–1299.
- Day-Vines, N. L., Wood, S. M., Grothaus, T., Craigen, L., Holman, A., Dotson-Blake, K., & Douglass, M. J. (2007). Broaching the subjects of race, ethnicity, and culture during the counseling process. *Journal of Counseling & Development*, 85(4), 401–409.
- DiAngelo, R., & Sensoy, Ö. (2014). Calling in: Strategies for cultivating humility and critical thinking in antiracism education. *Understanding and Dismantling Privilege*, 4(2), 190–203.
- Du Bois, W. E. B. (1903). *Souls of black folk: Essays and sketches*. Chicago: A. G. McClung.
- Fang, T. (2012). Yin Yang: A new perspective on culture. *Management and Organization Review*, 8, 25–50. <https://doi.org/10.1111/j.1740-8784.2011.00221.x>
- Farmer, P. E., Nizeye, B., Stulac, S., & Keshavjee, S. (2006). Structural violence and clinical medicine. *PLoS Medicine*, 3, (10), 1686–1691.
- Given, L. M. (2008). *The Sage encyclopedia of qualitative research methods*. Sage.
- Goffman, E. (1986). *Stigma: Notes on the management of a spoiled identity*. New York, NY: Simon & Schuster (Original work published 1963).
- Goins, R. T., Jones, J., Schure, M., Rosenberg, D. E., Phelan, E. A., Dodson, S., & Jones, D. L. (2014). Older adults' perceptions of mobility: A metasynthesis of qualitative studies. *The Gerontologist*, 55, 929–942. <https://doi.org/10.1093/geront/gnu014>
- Granerud, A., & Toft, H. (2015). Opioid dependency rehabilitation with the opioid maintenance treatment programme—A qualitative study from the clients' perspective. *Substance Abuse Treatment, Prevention, and Policy*, 10, 35. <https://doi.org/10.1186/s13011-015-00314>
- Grasswick, H. (2006). Feminist Social Epistemology. In: Zalta, E. N. (Ed.). *The Stanford Encyclopedia of Philosophy (Fall 2018 Edition)*. <https://plato.stanford.edu/archives/fall2018/entries/feminist-social-epistemology/>

- Grønnestad, T. E., & Sagvaag, H. (2016). Stuck in limbo: Illicit drug users' experiences with opioid maintenance treatment and the relation to recovery. *International Journal of Qualitative Studies on Health and Well-Being*, 11(1), 31992.
- Hatcher, A. E., Mendoza, S., & Hansen, H. (2018). At the expense of a life: Race, class, and the meaning of buprenorphine in pharmaceuticalized "care". *Substance Use & Misuse*, 53, 301–310. <https://doi.org/10.1080/10826084.2017.1385633>
- Hewell, V. M., Vasquez, A. R., & Rivkin, I. D. (2017). Systemic and individual factors in the buprenorphine treatment-seeking process: A qualitative study. *Substance Abuse Treatment, Prevention, and Policy*, 12, 3. <https://doi.org/10.1186/s13011-016-0085-y>
- Hodge, J. G. Jr., Gulinson, C. L., Barraza, L. F., Johnson, W. G., Hensley, D., & Augur, H. R. (2018). Exploring legal and policy responses to opioids: America's worst public health emergency. *South Carolina Law Review*, 70, 481.
- Jaeger, S. (2012). A geomedical approach to Chinese medicine: The origin of the Yin-Yang symbol. In Kuang, H. (Ed.), *Recent advances in theories and practice of Chinese medicine* (pp. 29–44). IntechOpen.
- Larney, S., Zador, D., Sindich, N., & Dolan, K. (2017). A qualitative study of reasons for seeking and ceasing opioid substitution treatment in prisons in New South Wales, Australia. *Drug and Alcohol Review*, 36, 305–310. <https://doi.org/10.1111/dar.12442>
- Larochelle, M. R., Bernson, D., Land, T., Stopka, T. J., Wang, N., Xuan, Z., & Walley, A. Y. (2018). Medication for opioid use disorder after nonfatal opioid overdose and association with mortality: A cohort study. *Annals of Internal Medicine*, 169, 137–145. 10.7326/M17-3107
- Lindgren, B.-M., Eklund, M., Melin, Y., & Graneheim, U. H. (2015). From resistance to existence—Experiences of medication-assisted treatment as disclosed by people with opioid dependence. *Issues in Mental Health Nursing*, 36, 963–970. <https://doi.org/10.3109/01612840.2015.1074769>
- Manchikanti, L., & Singh, A. (2008). Therapeutic opioids: A ten-year perspective on the complexities and complications of the escalating use, abuse, and nonmedical use of opioids. *Pain Physician*, 11(2, Suppl), S63–S88. <https://www.painphysicianjournal.com/current/pdf?article=OTgz&journal=42>
- Meer, N. (2019). W.E.B. DuBois, double consciousness and the 'spirit' of recognition. *The Sociological Review*, 67(1), 47–62.
- Redden, S. M., Tracy, S. J., & Shafer, M. S. (2013). A metaphor analysis of recovering substance abusers' sensemaking of medication-assisted treatment. *Qualitative Health Research*, 23, 951–962. <https://doi.org/10.1177%2F1049732313487802>
- Sandelowski, M., & Barroso, J. (2006). *Handbook for synthesizing qualitative research*. Springer.
- Sandelowski, M., Barroso, J., & Voils, C. I. (2007). Using qualitative metasummary to synthesize qualitative and quantitative descriptive findings. *Research in Nursing & Health*, 30(1), 99–111.
- U.S. Department of Health and Human Services. (2016). National pain strategy outlines actions for improving pain care in America. <https://www.hhs.gov/ash/aboutash/news/2016/national-pain-strategy-outlines-actions-improving-pain-care/index.htm>
- Vishwanath, T. P., Cash, P., Cant, R., Mummery, J., & Penney, W. (2019). The lived experience of Australian opioid replacement therapy recipients in a community-based program in regional Victoria. *Drug and alcohol review*, 38(6), 656–663.
- Wilson, M., Shaw, M. R., & Roberts, M. L. A. (2018). Opioid initiation to substance use treatment: "They just want to feel normal". *Nursing Research*, 67, 369–378. 10.1097/NNR.0000000000000298

Aimee Techau ORCID ID:

<https://orcid.org/0000-0002-5478-6737>

Emily Gamm ORCID ID:

<https://orcid.org/0000-0003-0953-5709>

Mia Roberts ORCID ID:

<https://orcid.org/0000-0001-9806-2813>

Lorraine Garcia ORCID ID:

<https://orcid.org/0000-0003-1013-3715>